Table 4. Recurring Qualitative Themes in Public-Facing Data Governance Disclosures.

Theme	Common disclosure pattern	Limitation	Relevance to neuromodulation consent
Data ownership and control	Data practices are framed using broad corporate legal terms.	Limited to no explanation of patient-centered rights or choices.	Patients may not be informed on who controls device-generated or therapy-related data
Secondary use	Data usage rights include research, analytics, product improvement, or business operations	Limited to no labeling of optional versus care-essential uses.	Patients may be unaware of therapy related data needs compared to nonessential uses that can be declined.
Third-party sharing	Broad language of sharing data including “affiliates, service providers, or partners”	Limited clarity of what neuromodulation-specific data are shared and why	Patients may not understand what health data leave the clinical/manufacturer ecosystem
Retention and de-identification	Retention described as “as long as necessary”; de-identification described generally	Few data-category-specific retention windows or deletion pathways	Long-lived implants create prolonged data stewardship obligations
Opt-out mechanisms	Opt-outs focused on marketing, cookies, or communications	Functional consequences for therapy data, remote monitoring, or cloud upload opt-outs are often unclear	Patients may not know what data privacy choices are available without compromising therapy